



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سميحه عبد الحفيظ موسى محمد

العنوان : حي الاسمرات

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 20-09-2026

رقم البطاقة : 25909052300447 رقم التليفون : 01069016575

العمر (تاريخ الميلاد) : 05-09-1959 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: سميرة عبد الحفيظ موسى محمد
National ID: 25909052300447



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سميحه
عبد الحفيظ موسى محمد
١٨ النيلسج - حي الاسمرات
المقطم - القاهرة

١٩٥٩/٠٩/٢٥ ٢٥٩٠٩٠٥٢٣٠٠٤٤٧
J10761157

DMS
مسعدري

٢٠٢٢/٠٤ ٢٥٩٠٩٠٥٢٣٠٠٤٤٧
ربة منزل
أفنى
مستلمة
أرملة

البطاقة سارية حتى ٢٠٢٩/٠٤/٠٥

١

03/09/2026

MDT FORM

Name: <u>Mossa Mohamed</u>	MRN: <u>6511</u>
Age: <u>67 Y</u>	ID: <u></u>
Menopausal Status: <u>POST MENOPUSAL</u>	Phone: <u>01141730312</u>
NRS SCORE: <u>0</u>	Marital Status: <u></u>
BMI: <u></u>	Family History: <u></u>
CO Morbidity: <u>DIABETIC HYPERTENSIVE</u>	Reason Of MDT: <u>NEW</u>

Clinical Data:

BX: IDC G2

ER 8/8 **PR** 8/8 **HER** AMPL **KI67** 25%

Sonomammography:
12 O'CLOCK MALIGNANT MASS 30X15 mm

Mets Workup:
CAP FREE
BONE SCAN FREE

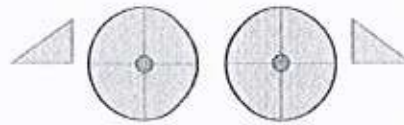
History:
T2 N1

Recommendations:
FOR POSSIBILITY OF NACT
CLIP INSERTION THEN BCS+SLNB

MDT Outcome & Plan
CLIP INSERTION NACT (TCHP X 6) THEN BCS+SLNB

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<u>د. يوسف روسي</u>	<u>د. أحمد شوقي</u>				

Patient Name	: الأستاذة / سميحة عبدالحفيظ موسى	Request No.	: 13003979
Patient No.	: 25909052300447	Request Date	: 15/12/2025
Path No.	: 3872/25	Verification Date	: 25/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 66 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Left breast 12-1 o'clock mass.

Nature of specimen

Tru cut biopsy, collected on 11:55 a.m.

Gross examination

Multiple(5) grayish white tissue cores ranging in size from 0.5 up to 1.5 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
- * DCIS: Focal, solid , cribriform , low grade .
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification : Positive

Diagnosis

Left breast 12 -1 o'clock mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, HER2, Ki67)

Signature

Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-Shafeey

Verified by :

Dr. Heba Hasan

Printing Date : 28/12/2025 13:40:39

Page 1 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :	الأستاذة / سميرة عبد الحفيظ موسى	Request No. :	13003979
Patient No. :	25909052300447	Request Date :	15/12/2025
Path No. :	3872/25	Verification Date :	25/12/2025
Sex :	Female	Financial Category :	جوستاف روسي - مبادره
Age :	66 Y	Department :	Out Patient
Referring Doc. :		IHC No. :	
		Registered by :	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 98% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8).
- Her2: Moderate complete membrane reaction in >10 % tumor cells (score 2+).
- SISH :
- Average HER2 COPY NO.4.5
- HER2 /CEP17 ratio :3.2
- Ki67: 25% .

Conclusion:
Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Positive(score 8/8).
- Her2: Equivocal (score 2+).
- SISH :Amplified .

Verified by :
Dr.Heba Hasan



Patient Name الأستاذ / سميرة عبد الحفيظ موسى
Patient No. 25909052300447
Path No. 3872/25
Sex Female
Age 66 Y
Referring Doc.

Request No. 13003979
Request Date 15/12/2025
Verification Date 25/12/2025
Financial Category جوسف روسي - مبادره
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

- Ki67: 25 %
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibraim Ghoneim
Dr. Heba El-Shafeey

Verified by :
Dr. Heba Hasan



Patient ID: 93420	Patient Name English: Samahah Abd Alhafiz Mosa Mohamed
Age: 66 years old	2025/12/02 02:23:00 PM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: left palpable lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated normal parenchyma
- No solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LN

• **Left breast**

- Scattered fibro glandular densities (ACR density B).
- Upper central irregular dense spiculated mass with associated distortion
- **Ultrasound** : 12:00 zone B irregular spiculated hypoechoic mass , measures 3x1.5cm located 1.4cm from the skin .(index lesion)
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Level I indeterminate lymphnode shows non uniform cortical thickness up to 4.4mm with preserved hilum

Impression:

Left breast (BIRADS5): 12:00 malignant mass for US guided biopsy

Right breast (BIRADS 1): normal

Dr. Marwa Ramz, MD, FRCR

*for MACT followed by
CBS & SNLB*

Patient ID: 93793	Patient Name English: Samihah Abdalhafiz Mosa
Age: ٤٠ سنة ٤ شهر	2026/03/16 10:05:05 AM :Report Date

Women's Health Outreach Program

Clinical summary: Pathologically proven Left breast invasive ductal grade II (+ve ER, +ve PR, +ve HER2 and Ki67= 25 %) for clip insertion

Ultrasound guided clip insertion

Left breast

- Under complete aseptic condition & ultrasound guidance one clip insertion was done for left breast malignant mass lesion at 12 o'clock the clip were inserted at center lesion and this confirmed by mammographic CC & ML

Best Regards,

Dr. Marwa M.Hassan

Name : Samihah Abdel Hafiz Musa

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

Date : 21-01-2026 09:44

Medical No. : 299607

Ins.Company : تعاقد مستشفى

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 66-year-old lady, with history of left breast cancer.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Name : SAMIHA ABD AL HAFIZ MOSA

Branch : Portable

Ref.Physician Prof : CT CAP+C

Date : 24-01-2026 04:10

Medical No. : GRI-95181

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -

Reported case with breast cancer

1. Chest:

- Left breast spiculated mass measures 4.1 x 3.6 cm skin thickening reaching 6mm.
- Left axillary level I and II prominent lymph nodes .
- No sizable pulmonary nodules or masses .
- No hilar or mediastinal lymph nodes, .
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

1. Abdomen and pelvis:

- The liver is enlarged in size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen , small splenule noted .
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

2. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***No definite pulmonary , hepatic or bony metastatic deposits .***

Dr/Reham Khedr ,,,MD
16/11/2025 8:00 AM

ECHO REPORT

NAME: *Elie*

ID NUMBER: *1*

DATE: 26/4/202

SEX: AGE:

EF	65%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3.3-7cm
LVEDD	2.3	2.4cm	LA	3.6	2.4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *Normal*

LV Wall Thickness *Normal*

LV Diastolic Function *GI*

SWMA *No RWA*

LA and Ao Root Dimensions *Normal*

Mitral Valve *N*

Aortic Valve *N*

Tricuspid Valve *N*

Pulmonary Valve

Right Side and Systolic Function *TAPSE: 2.3*

Intracardiac Masses and Effusion

DR. *Elie*

Patient ID: 90420	Patient Name English: Samihah Abd Alhefiz Mosa Mohamed
Age: 66 years old	2025/12/02 02:23:00 PM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: left palpable lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated normal parenchyma
- No solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

• Left breast

- Scattered fibro glandular densities (ACR density B).
- Upper central irregular dense spiculated mass with associated distortion
- Ultrasound : 12:00 zone B irregular spiculated hypoechoic mass , measures 3x1.5cm located 1.4cm from the skin .(index lesion)
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Level I indeterminate lymphnode shows non uniform cortical thickness up to 4.4mm with preserved hilum

Impression:

Left breast (BIRADS5): 12:00 malignant mass for US guided biopsy

Right breast (BIRADS 1): normal

Dr. Marwa Ramz, MD, FRCR